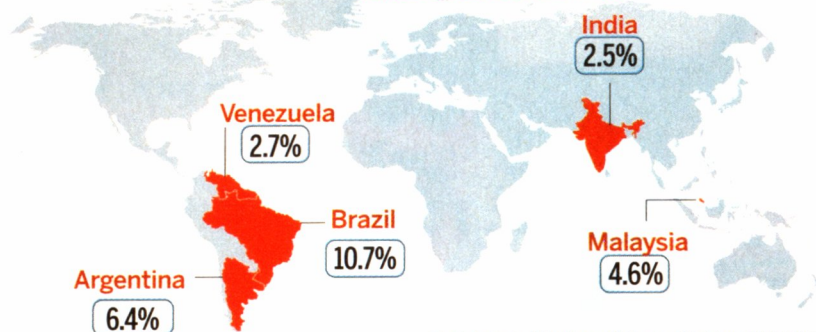


Top 5 by fatality rate

In a strange twist, higher infection rates do not mean a higher death toll

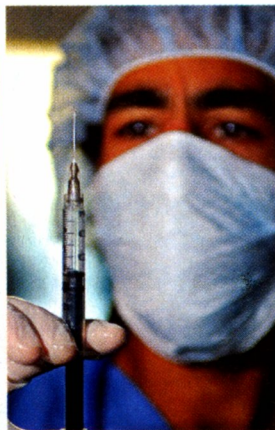


Graphics by: Prashant Chaudhary and Madhumangal Singh / www.indiatodayimages.com

GLOBAL FLU TRACKER

208 countries report 14,142 deaths.
52% infected are between age 16 and 44.
462 people per million infected in Australia. It's the highest infection rate per capita.
47 million people infected in the US, considered the largest infection hub in the world.
600 million vaccine doses stockpiled by the US, 130 million by the UK, 94 million in France—more than one for every citizen

Sources: WHO, ECDC, CDC



appears limited in all three. "There's an unfortunate lack of clarity in the national policy," he says. "The Government has done as much as they could, with the best of intentions. But that's not enough to pay the right dividends." To the man who first reported oral polio vaccine efficacy problems in India and was the first to identify individuals with HIV infection in the country, a serious lack of attention to influenza has resulted in this. "We never developed a policy on the study of epidemiology and interventions of endemic influenza, including the need for vaccination," he points out. The Pasteur Institute of India in Coonoor, for instance, had first isolated the H2N2 and H3N2 strains during the pandemics of 1957 and 1968.

But that knowledge, which could have proved precious now, was left to gather dust, he explains.

The economics of swine flu vaccine also doesn't work in India's favour. The Government committed Rs 333 crore to check the pandemic. A worthy amount, but certainly not in the same league as the global majors that are driving free, mass vaccination campaigns. The UK Government, for instance, spent more than £100 crore in preparation for a flu pandemic and is ready with 130 million doses—more than one for every citizen. The US started with \$100 crore and kept \$750 crore more available in emergency preparedness funds. No wonder, the country has stockpiled 600 million doses of vaccine. With imported vac-

cines at Rs 300-500 and indigenous varieties at Rs 50-100 per vial, mass vaccination is hardly practicable for India. "Our philosophy of high volume and low cost would definitely be the real answer to providing affordable vaccine," adds Cyrus Poonawalla, CMD, Serum Institute. "But that depends to a large extent on both the World Health Organization (WHO) and the Government to lift, support, procure and stockpile the vaccine."

There are some troubling issues with the pandemic in India that refuse to go away: India may not have seen a raging spread compared to the global figures, but the death toll has been twice the world rate (1.9: 0.82 per cent). Also, 75 per cent of the infection has been heavily concentrated in the more affluent urban centres. Surprisingly, that's not because of the presence of unhygienic slums in cities. Deo's study, for instance, shows that slum dwellers of Pune (42 per cent of the 4 million population) account for just 4.5 per cent of the deaths. The country is witnessing a spurt in new pockets—Maharashtra, Rajasthan, Karnataka and Gujarat—from mid-December.

Will it spin out of control during the next flu season of July-August? The answer, like the capricious virus, is blowing in the wind. The novel virus created panic in public mind from the very beginning. With the WHO now admitting that its seriousness may have been overestimated (and with reports on "conflict of interest" between its committee members and pharma majors doing the rounds) the confusion is fast turning into a "pandemic double think": some rubbishing it as a "hoax", others waiting to get immunised, and still others determinedly refusing to get a vaccine whose efficacy is still untested. The road ahead is uncertain, but definitely not short. The health sector can start by thanking its stars that the pandemic proved to be mild. The need of the hour is to learn from the experience and shore up a shaky public health infrastructure to prevent a future disaster. The next time, we might not be so lucky. ■



"OTHER COUNTRIES HAVE A BACKGROUND OF REGULARLY USING SEASONAL FLU VACCINE. SO IT'S MUCH EASIER FOR THEM TO SWITCH TO A NEW STRAIN."

Dr T. JACOB JOHN, Professor Emeritus, Clinical Virology, CMC, Vellore